Ideation Phase
Define the Problem Statements

Date	28 October 2026
Team ID	Codesian
Project Name	Health360: Comprehensive Healthcare Risk & Disease Analytics
Maximum Marks	2 Marks

Healthcare Administrators, Clinical Risk Managers, and Physicians:

Problem Statement (PS): PS-1

I am	Describe customer with 3–4 key characteristics — who are they?	Healthcare Administrator & Chief Medical Officer
I'm trying to	List their outcome or 'job' — what are they trying to achieve?	Screen and identify early-onset hypertension across patient age brackets.
but	Describe what problems or barriers stand in the way — what bothers them most?	Traditional screening focuses primarily on senior demographics, missing early hypertension trends in younger populations.
because	Enter the root cause of why the problem or barrier exists — what needs to be solved?	Clinical data is siloed and lacks age-group segmented visualization tools.
which makes me feel	Describe the emotions that result from experiencing the problems or barriers.	Concerned that unmonitored stage-1 hypertension in teenagers and young adults will lead to severe long-term cardiovascular complications.

Problem Statement (PS): PS-2

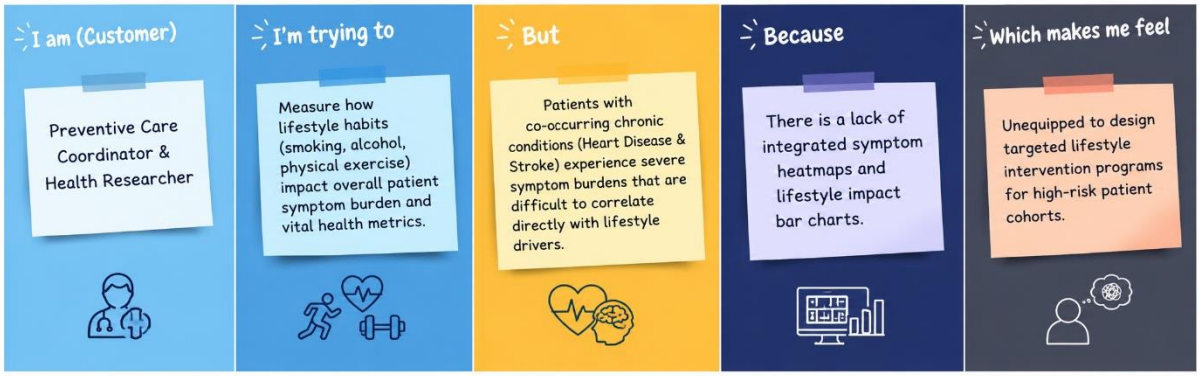
I am	Describe customer with 3–4 key characteristics — who are they?	Clinical Risk Manager & Primary Care Physician
I'm trying to	List their outcome or 'job' — what are they trying to achieve?	Stratify patients into Dynamic Risk Tiers (High, Moderate, Low Risk) based on health vitals.
but	Describe what problems or barriers stand in the way — what bothers them most?	Single-variable risk evaluations fail to capture complex, non-linear interactions between Glucose levels, BMI, and chronic conditions.
because	Enter the root cause of why the problem or barrier exists — what needs to be solved?	Existing diagnostic tools do not offer interactive multi-variable scatter plot matrices.
which makes me feel	Describe the emotions that result from experiencing the problems or barriers.	Frustrated by the inability to quickly spot high-risk patient clusters during routine clinical reviews.

Problem Statement 3 (PS-3)

I am	*Describe customer with 3–4 key characteristics — who are they?*	Preventive Care Coordinator & Health Researcher
I'm trying to	*List their outcome or 'job' — what are they trying to achieve?*	Measure how lifestyle habits (smoking, alcohol, physical exercise) impact overall patient symptom burden and vital health metrics.
but	*Describe what problems or barriers stand in the way — what bothers them most?*	Patients with co-occurring chronic conditions (Heart Disease & Stroke) experience severe symptom burdens that are difficult to correlate directly with lifestyle drivers.
because	*Enter the root cause of why the problem or barrier exists — what needs to be solved?*	There is a lack of integrated symptom heatmaps and lifestyle impact bar charts.
which makes me feel	*Describe the emotions that result from experiencing the problems or barriers.*	Unequipped to design targeted lifestyle intervention programs for high-risk patient cohorts.

Reference: https://miro.com/templates/customer-problem-statement/

Example:



Problem Statement (PS)	**I am (Customer)**	**I'm trying to**	**But**	**Because**	**Which makes me feel**
PS-1	Healthcare Administrator & Chief Medical Officer	Screen and identify early-onset hypertension across patient age brackets.	Traditional screening focuses primarily on senior demographics, missing early hypertension trends in younger	Clinical data is siloed and lacks age-group segmented visualization tools	Concerned that unmonitored stage-1 hypertension in younger patients (Teenagers and Young Adults) will lead to severe long-term cardiovascular complications.

			populations		
PS-2	Clinical Risk Manager & Primary Care Physician	Stratify patients into Dynamic Risk Tiers (High, Moderate, Low Risk) based on health vitals.	Single-variable risk evaluations fail to capture complex, non-linear interactions between Glucose levels, BMI, and chronic conditions.	Existing diagnostic tools do not offer interactive multi-variable scatter plot matrices.	Frustrated by the inability to quickly spot high-risk patient clusters during routine clinical reviews.